Sutter Health
Sutter Gould
Medical Foundation

FAX

Date:

To: Golden Bear **From:**

Dept.: Northern Regional **Dept.:** Pain Management

Fax No.: 209-222-6182 **Fax No.:** 209-342-6671

Phone No.: **Phone No.:** 209-550-4762

Cc:

No. Pages: (including coversheet)

Comments:

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GOLDEN BEAR PHYSICAL THERAPY

REHABILITATION & WELLNESS

Patient Name: Brice, Fred
Legal Name: Brice, Fred
MRN: 67554590 CSN: 1532036265
Date of Birth: DOB: 8/7/1969 Sex: M ENC: 217860662
Svc Date/Time: 5/04/26 10:30 AM
Svc Prov: SAWERIS, YOUSSEF FAYEZ RAGHEB
Svc Dep: SUTTER PAIN MANAGEMENT -- MODESTO

Patient Telephone: _____

Physician Name: _____

Physician Telephone: _____

Diagnosis: M54.16, M47.812, M54.12, 298.890

☒ Acute ☐ Chronic

Special Instructions: _____

TREATMENT:

☑ EVALUATE AND TREAT

☐ POST-SURGICAL REHABILITATION

☒ PHYSICAL THERAPY

☐ AQUATIC THERAPY*

FREQUENCY:

☐ 3x ☑ 2x ☐ 1x ☐ Daily

DURATION: 6 weeks

Physician Signature Date

DISCOVER HOW TO RECOVER AT ONE OF OUR MANY LOCATIONS!

NORTHERN REGIONAL LOCATIONS

El Dorado, San Joaquin and Calaveras Counties

northernfax@goldenbearpt.com | Fax: 209.222.6182

☐	2222 Francisco Dr., Ste 120, El Dorado Hills 95762	P: 916.235.4071
☒	1519 W. Yosemite Ave., Manteca 95337	P: 209.823.3322
☐	1019 S. Main St., Manteca 95337	P: 209.624.1002
☐	855 S. Tracy Blvd., Tracy 95376	P: 209.627.1383
☐	1222 W. Colony Ave., Ste 130, Ripon 95366	P: 209.624.1288
☐	1906 Vista Del Lago Dr., Ste A, Valley Springs 95252	P: 209.772.0848
☐	2339 W. Hammer Ln., Ste K, Stockton 95209	P: 209.623.2500
☐	631 S. Ham Ln., Ste B, Lodi 95242	P: 209.368.7433
☐	210 W. Pine St., Lodi 95240	P: 209.365.7867

CENTRAL VALLEY REGIONAL LOCATIONS

Stanislaus County

referrals1@goldenbearpt.com | Fax: 209.622.4159

☐	1335 Coffee Rd., Ste 200, Modesto 95355	P: 209.576.1946
☐	*4318 Spyres Way, Modesto 95356	P: 209.576.0710
☐	1533 Oakdale Rd., Ste B, Modesto 95355	P: 209.554.5233
☐	146 N. Maag Ave., Ste B, Oakdale 95361	P: 209.322.2140
☐	2603 Patterson Rd., Ste 8, Riverbank 95367	P: 209.896-1870
☐	2908 E. Whitmore Ave., Ste E, Ceres 95307	P: 209.622.4149
☐	2119 E. Hatch Rd, Ste A, Modesto 95351	P: 209.812.3055

SOUTHERN REGIONAL LOCATIONS

Stanislaus, Merced Counties

southernfax@goldenbearpt.com | Fax: 209.222.6185

☐	* 3800 Geer Rd., Ste 101, Turlock 95382	P: 209.667.1729
☐	2303 Geer Rd., Turlock 95382	P: 209.585.4100
☐	1700 Keystone Pacific Pkwy., C2, Patterson 95363	P: 209.895.4206
☐	* 3184 Collins Dr., Merced 95348	P: 209.230.5289
☐	1229 Commerce Ave., Atwater 95366	P: 209.683.1386
☐	* 1400 Mercey Springs Way, Ste I, Los Banos 93635	P: 209.710.1011
☐	468 Winton Pkwy., Livingston 95334	P: 209-354-8274
☐	4080 N. Cedar Ave., Fresno 93726	P: 559.222.7497
☐	1586 Constitution Blvd., Salinas 93905	P: 831.276.0565
☐	1916 N. Main St., Salinas 93906	P: 831.276.0560

** Aquatic Therapy Location*

#DiscoverHowToRecover

UPDATED: JUNE 2022

Sutter Health

SGCV MODESTO 1401 SPANOS COURT
1401 Spanos Court
Modesto, CA 95355-2816

COMM METHOD	ETHNICITY	APPT DATE/TIME
Mobile	NON HISPANIC	5/04/2610:30 AM SAWERIS, YOUSSEF FAYEZ R* SUTTER PAIN MANAGEMENT - MODESTO 1401 SPANOS COURT, SUITE 130 MODESTO CA 95355-2816
RACE	PATIENT TYPE	
WHITE/CAUCASIAN		

ACCOUNT #	PT. CLASS	SVC	UNIT/ROOM/BED	ACCOM	ADMIT TYPE	SOURCE	MEDICAL RECORD #
IP ONLY	IP ONLY	IP ONLY	IP ONLY	None	IP ONLY	IP ONLY	57554590

AGE	BIRTHDATE	SEX	MARITAL STATUS	RELIGION	PRIMARY LANGUAGE	INTP NEEDED	EPIC ID#
56 Y	8/7/1969	M	MARRIED	CHRISTIAN	ENGLISH	NO	EEID #: 6195737

PATIENT NAME/ADDRESS/PHONE#

BRICE, FRED
940 NORDIC PL

MANTECA CALIFORNIA 95336
Pref #: 209-812-5577 Cell #: Mobile phone #: 209-812-5577
EMAIL: FKBRICE96@YAHOO.COM

PATIENT - EMPLOYER NAME/ADDRESS/PHONE# **OCCUPATION:**

2820 N WESTERN PKWY

SANTA CLARA, CALIFORNIA 95051
408-881-2679

GUARANTOR - NAME/ADDRESS/PHONE **RELATIONSHIP TO PATIENT**

BRICE,FRED Self
940 Nordic Pl

Manteca, CA 95336

SEX: Male DOB: 8/7/1969

HOME PHONE: 209-812-5577

GUARANTOR - EMPLOYER NAME/ADDRESS/PHONE# **OCCUPATION:**

500 LOUISE AVE;LATHROP
LATHROP, California 95330
209-812-5577

EMERGENCY CONTACT ONE /ADDRESS/PHONE# **RELATIONSHIP TO PT:**

BRICE,KELLEY SPOUSE
940 NORDIC PL

MANTECA, CALIFORNIA 95336
HOME PHONE: 209-603-4404
WORK:
MOBILE#

EMERGENCY CONTACT TWO /ADDRESS/PHONE# **RELATIONSHIP TO PT:**

HOME #:
WORK #:
MOBILE#

1st COVERAGE/NAME/ADDRESS/PHONE#

PGBA/TRICARE WEST
PO BOX 202160
FLORENCE, SC 29502-2160

ID #: 00637091900 PHN: 888-874-9378

GROUP #: RFL #:

2nd COVERAGE/NAME/ADDRESS/PHONE#

- No Secondary Coverage -

ID #: PHN:

GROUP #:

3rd COVERAGE/NAME/ADDRESS/PHONE#

- No Tertiary Coverage -

ID #: PHN:

GROUP #:

4th COVERAGE/NAME/ADDRESS/PHONE#

- No Quartinery Coverage -

ID #: PHN:

GROUP #:

PHYSICIAN INFO:

ENC PROVIDER: SAWERIS, YOUSSEF FAYEZ R*
SEC PROVIDER: SAWERIS, YOUSSEF FAYEZ R*
REFERRING: Chowdhury, Sukalpa, MD
PCP: Chowdhury, Sukalpa, MD

ADMITTING DIAGNOSIS / CHIEF COMPLAINT:

Brice, Fred

MRN: 57554590

Office Visit 5/4/2026
Status: Open
Sutter Pain Management - Modesto

Provider: Saweris, Youssef Fayez Ragheb, MD (Anesthesiology)
Primary diagnosis: Lumbar radiculopathy
Reason for Visit: Referred by Chowdhury, Sukalpa, MD

Progress Notes

Unsigned

Saweris, Youssef Fayez Ragheb, MD (Anesthesiologist) • Anesthesiology

CHRONIC PAIN NEW

PATIENT EVALUATION

Referring Physician: Sukalpa Chowdhury, MD
1026 Lifestyle St
MANTECA, CA 95337

Chief Complaint: lower back pain

SUBJECTIVE:

Patient/Parent/Legal Guardian gave verbal authorization to the use of ambient listening technology and third party audio recording to assist with this clinical encounter.

Fred Brice is a 56 year old male who presents to The Sutter Health Pain Management Center for the evaluation of lower back pain.

History of Present Illness

Does the pain radiate? {YES/NO:63}
Distribution: {PMFV DISTRIBUTION:88123}
Is the pain related to recent trauma? {YES/NO:63}
Previous pain treatments: see above
Occupation: {OCCUPATIONAL STATUS:36969}

Patient {DENIES/REPORTS:155682} {RED FLAGS:155679}

Pain Medications:
Reviewed

PDMP report: Not reviewed}

Pain medications reviewed: Yes

Pain Procedures: ***

Medical History

Diagnosis	Date	Comment	Source
LUMBAR SPONDYLOSIS NO MYELOPATHY	09/07/2012		
Diabetes mellitus (CMS/HCC)			
Hyperglycemia			

Past Surgical History:

Procedure	Laterality	Date
• COLONOSCOPY		2013
• HX BACK SURGERY NOS* (NO BILL) *L4-L5*		
• HX LAPAROSCOPIC CHOLECYSTECTOMY *Cholecystectomy, laproscopic*		1999

Social Hx not Including Intimate Partner Violence

Social History

Socioeconomic History

• Marital status:	Married
Spouse name:	Not on file
• Number of children:	2
• Years of education:	Not on file
• Highest education level:	Not on file

Occupational History

- Not on file

Tobacco Use

• Smoking status:	Never
• Smokeless tobacco:	Never

Vaping Use

• Vaping status:	Not on file

Substance and Sexual Activity

• Alcohol use:	Yes
Comment: rarely	
• Drug use:	No
• Sexual activity:	Yes
Partners:	Female

Other Topics	Concern
• Military Service	Not Asked
• Blood Transfusions	Not Asked
• Caffeine Concern	No
• Occupational Exposure	Not Asked
• Hobby Hazards	Not Asked
• Sleep Concern	No
• Stress Concern	Yes
Comment: has an autistic son	
• Weight Concern	No
• Special Diet	No
• Back Care	No
• Exercise	Not Asked
• Bike Helmet	Not Asked
• Seat Belt	No
• Self-Exams	Not Asked

Social History Narrative

Does patient have a history of:

Patient/Family hx of Malignant Hyperthermia: No
Cardiac Implants: No

Active communicable disease (examples: MRSA, TB, shingles, chickenpox VRE): No
On Dialysis: No
Currently on Supplemental Oxygen: No
Anaphylaxis reaction to latex: No

History of Anesthesia Reaction: No
Loop Recorder: No
Admission to hospital within (30 days) or ECF (6 months): No
Currently using a CPAP for sleep apnea: No
History of blood clots: No
Allergy to metal:No

Is patient diabetic or on medication for pre-diabetes?. Yes, Type II diabetes

Are you currently under the care of any of the following specialists?: None

Social Drivers of Health From This Encounter

Core SDOH Screening Questions: Not on file
Alcohol Use: Not on file
Tobacco Use: Low Risk (5/4/2026)
Patient History
- Smoking Tobacco Use: Never
- Smokeless Tobacco Use: Never
- Passive Exposure: Not on file

Food Insecurity: Not on file
Financial Resource Strain: Not on file
Physical Activity: Not on file
Transportation Needs: Not on file
Social Connections: Not on file
Stress: Not on file
Depression: Not on file
Utilities: Not on file
Education: Not on file
Housing Stability: Not on file
Family History
Family History

Problem	Relation	Name	Age of Onset
• Blood disorder	Mother		
• Cancer	Mother		
• Coronary Artery Disease	Father		
• Hypertension	Father		
• Diabetes type II	Father		
• Multi Sclerosis	Sister		
• Cerebrovascular Disease/Stroke	Maternal Grandmother		
• Colon Cancer	Maternal Grandfather		
• Lung Cancer	Maternal Grandfather		
• COPD	Maternal Grandfather		
• Colon Cancer	Maternal Aunt		

• Lung Cancer	Maternal Aunt
• Lung Cancer	Maternal Aunt
• Stomach Cancer	No Significant Family History

Review of patient's allergies indicates no known allergies.

Current Medications

Current Outpatient Medications

Medication	Sig
• etodolac (LODINE) 500mg Tab	Take one Tab by mouth twice daily
• folic acid 1mg Tab	Take one Tab by mouth daily
• methotrexate 2.5mg Tab	Take four Tabs by mouth once weekly
• omeprazole DR (PRILOSEC) 40mg DR Cap	Take one Cap by mouth daily 30 minutes before breakfast
• atorvastatin (LIPITOR) 20mg Tab	Take one Tab by mouth daily Dose changed
• gabapentin (NEURONTIN) 300mg Cap	Take one Cap by mouth twice daily
• diclofenac sodium (VOLTAREN) 1% Gel	four g to affected area(s) four times daily (max: 16 grams/joint/day; 32 grams/day body max)
• buPROPion 24Hr-XL (WELLBUTRIN XL) 300mg Tab	Take one Tab by mouth daily Dose change
• Continuous Blood Glucose Sensor (FREESTYLE LIBRE 2 SENSOR)	Change every 14 days
• empagliflozin (JARDIANCE) 25mg Tab	Take one Tab by mouth every morning Dose change
• tadalafil (CIALIS) 10mg Tab	Take one Tab by mouth as directed Take 30 minutes prior to anticipated sexual activity. Max dose is 20mg in 24 hours.
• sildenafil (VIAGRA) 100mg Tab	Take one Tab by mouth as directed Take 30 minutes to 4 hours before sexual activity. Max dose is 100mg in 24 hours.
• FreeStyle Lancets MISC Lancets	Check blood sugar three times a day
• pioglitazone (ACTOS) 15mg Tab	Take one Tab by mouth daily Dose change
• tirzepatide (MOUNJARO) 5mg/0.5mL Auto Injector	Administer 0.5 mL subcutaneously once weekly
• [START ON 5/12/2026] tirzepatide (MOUNJARO) 7.5mg/0.5mL Auto Injector	Administer 0.5 mL subcutaneously once weekly Start 7.5 mg weekly after completing 4 weeks of 5 mg weekly

No current facility-administered medications for this visit.

REVIEW OF SYSTEMS:
GENERAL: No weight loss, malaise or fevers.
HEENT: Negative for frequent or significant headaches.
NECK: Negative for lumps, goiter, and significant neck swelling.
RESPIRATORY: Negative for cough, wheezing or shortness of breath.
CARDIOVASCULAR: Negative for chest pain, leg swelling or palpitations.
GI: Negative for abdominal discomfort, blood in stools or black stools or change in bowel habits.
MUSCULOSKELETAL: See HPI.
SKIN: Negative for lesions, rash, and itching.
PSYCH: Negative for sleep disturbance, mood disorder and recent psychosocial stressors.
HEMATOLOGY/LYMPHOLOGY: Negative for prolonged bleeding, bruising easily or swollen nodes.
NEURO: No history of headaches, syncope, paralysis, seizures or tremors.
ENDOCRINE: *** DM I/DMII
All other reviewed and negative other than HPI.
OBJECTIVE:

Vitals:

	05/04/26 1033
BP:	(P) 131/67

PHYSICAL EXAMINATION:
General appearance: Well appearing, in no acute distress, alert.
Psych: Mood and affect appropriate.
Skin: Skin color, texture, turgor normal, no rashes or lesions.
Head/face: Normocephalic, atraumatic.
Neck: **Inspection:** Neck is symmetric without visible masses, swelling, erythema, or deformity. No scars or abnormal posture noted.
Palpation : *** tenderness over cervical paraspinal muscles or spinous processes.
Range of Motion (ROM): Full and painless in flexion, extension, lateral bending, and rotation. ***
Special/Abnormal findings: neg Spurling test, neg hoffman test ***
Pulm: unlabored
GI: Not examined
Back: **Inspection:** Normal lumbar alignment without scoliosis, kyphosis, or lordotic flattening.
Palpation : *** tenderness over lumbar paraspinal muscles, *** tenderness to palpation over bilateral sacroiliac joints.
Range of Motion (ROM): Full and symmetric in flexion, extension, lateral bending, and rotation. ***
Special/Abnormal findings: Straight leg raising in the sitting and supine positions is negative to radicular pain. *** Patrick's, *** Gaenslen's, *** FABER
Extremities: ROM is full and pain free without obvious instability or laxity in all four extremities.
Neuro: Bilateral upper and lower extremity coordination symmetric. No gross abnormality. No obvious focal deficits.
Gait: Normal.

Physical Exam

Recent imaging: I personally reviewed available images
Lumbar spine MR:

FINDINGS:

Alignment: Normal alignment of the spine.

Vertebral body heights and marrow: Multilevel lumbar spondylosis with osteophytes, facet arthropathy, and endplate marrow degenerative changes. Left L4-L5 laminotomy. Background of congenitally shortened pedicles. Otherwise the vertebral body heights and marrow signal are unremarkable.

Conus medullaris: Normal, terminating at L1.

Soft tissues: Fatty infiltration of the paraspinal musculature.

At T12-L1: No significant spinal canal stenosis or neuroforaminal narrowing.

At L1-L2: No significant spinal canal stenosis or neuroforaminal narrowing.

At L2-L3: Disc bulge, facet arthropathy, and ligamentum flavum hypertrophy contributing to mild bilateral neural foraminal narrowing and slight effacement of the ventral CSF space.

At L3-L4: Disc bulge, facet arthropathy, and ligamentum flavum hypertrophy contributing to borderline mild spinal canal stenosis and mild right neural foraminal narrowing.

At L4-L5: Disc bulge with central protrusion/scarring and facet arthropathy contribute to effacement of the

ventral CSF space as well as moderate right and mild to moderate left neural foraminal narrowing. Left facet synovial cyst.

At L5-S1: Disc bulge with left paracentral disc protrusion and facet arthropathy contribute to effacement of the left lateral recess as well as moderate to severe bilateral neural foraminal narrowing.

IMPRESSION:
1. Degenerative changes of the lumbar spine with multilevel neural foraminal narrowing, most pronounced at L5-S1.
2. Persistent left paracentral disc protrusion at L5-S1 with effacement of the left lateral recess.

Electronically Signed by: Kristopher de Ga, MD 11/7/2025 1:06 PM

Mr SI joints:
FINDINGS:
No ankylosis. No periarticular edema or sclerosis.

Mild cortical irregularity and osteophyte formation.

Lumbar spondylosis with intervertebral disc extrusions at L4-L5 and L5-S1.

IMPRESSION:
1. Sacroiliac joint degenerative change without evidence of sacroiliitis.
2. Lumbar spondylosis.

Electronically Signed by: Avik Patel, DO 2/27/2026 11:08 AM

Cervical spine MRI:
FINDINGS:

Examination is partially limited by artifact.

Alignment: Rotary curvature of the spine. There is up to grade 1 listhesis at multiple levels.

Vertebrae/Discs: There is multilevel disc desiccation, osteophytes, Schmorl's nodes, and incidental hemangiomas.

Cervical Spinal Cord: There is abnormal cord signal seen at C5-C6 series 5 image 21, series 3 image 10.

C2-3: Disc osteophyte complex, facet arthropathy, ligamentum flavum hypertrophy. Moderate spinal canal stenosis. Severe neuroforaminal stenosis bilaterally.

C3-4: Disc osteophyte complex, mild bilateral uncovertebral hypertrophy, facet arthropathy. Moderate spinal canal stenosis. Severe bilateral neuroforaminal stenosis.

C4-5: Disc osteophyte complex, ligamentum flavum hypertrophy, facet arthropathy. Severe spinal canal stenosis. Severe bilateral neuroforaminal stenosis.

C5-6: Disc osteophyte complex, right uncovertebral hypertrophy, facet arthropathy. Severe spinal canal stenosis. Ventral cord deformity. Severe bilateral neuroforaminal stenosis.

C6-7: Disc osteophyte complex, mild facet arthropathy. Mild to moderate spinal canal stenosis. Moderate bilateral neuroforaminal stenosis.

C7-T1: Small disc osteophyte complex, facet arthropathy. Mild spinal canal stenosis. Moderate to severe right and moderate left neuroforaminal stenosis.

There are some nonspecific lymph nodes which are not enlarged by size criteria. There is a nodule in the partially visualized right thyroid gland, recommend follow-up ultrasound. Mild sinus disease in the partially [illegible] nasal sinuses.

IMPRESSION:

[illegible] degenerative changes with up to severe spinal canal and neuroforaminal stenosis at multiple [illegible] detailed above.

There is abnormal cord signal at C5-6 which may reflect cord edema and/or myelomalacia.

[illegible] for right thyroid nodule.

Study flagged for significant findings, ordering provider will be notified by radiology department.

Electronically Signed by: Vinish K Mathur, MD 1/21/2026 12:41 AM



Results

LABORATORY:

CBC:

[illegible]

	Value	Date/Time
WBC	7.9	04/16/2026 09:29 AM
RBC	5.14	04/16/2026 09:29 AM
HGB	16.0	04/16/2026 09:29 AM
HCT	46.9	04/16/2026 09:29 AM
PLT	182	04/16/2026 09:29 AM

CMPA:

[illegible]

	Value	Date/Time
BUN	25	04/16/2026 09:29 AM
CREATININE	1.07	04/16/2026 09:29 AM
NA	141	04/16/2026 09:29 AM
K	4.4	04/16/2026 09:29 AM
CO2	25	04/16/2026 09:29 AM
CL	106	04/16/2026 09:29 AM
GLU	141	04/16/2026 09:29 AM

[illegible]	8.8	04/16/2026 09:29 AM
[illegible]	3.8	04/16/2026 09:29 AM
[illegible]	0.9	04/16/2026 09:29 AM
PROT	7.2	04/16/2026 09:29 AM
[illegible]	85	04/16/2026 09:29 AM
ALT	42	04/16/2026 09:29 AM
AST	24	04/16/2026 09:29 AM

[illegible] INFORMATION:
ASA Class: {ASA CLASS IP SH:112416}
Mallampati Score: {number 1-4:109994}

EMG/NCV:
***

Current Anticoagulant Therapy: {YES/NO:63}
[illegible] **YES/NO:87019}**
Physical Therapy/Home Exercise: {YES/NO:63}

Assessment & Plan

Assessment & Plan**

(M54.16) Lumbar radiculopathy (primary encounter diagnosis)
(M47.812) Cervical spondylosis
(M54.12) Cervical radiculopathy
(Z98.890) S/P laminectomy

- The patient was educated to go to the emergency department if there is any change in bowel or bladder control, any difficulty with gait/balance or any escalating numbness or tingling or any new loss of sensation or any upper or lower extremities weakness.
- Counseled patient regarding the importance of{Pain Counseling:155684}. Patient was counseled about the risk of over the counter medications.

- anticoagulation and antiplatelet management: ***
Patient was counseled regarding the bleeding risks associated with interventional procedures and the importance of proper medication management. Patient was informed that certain medications/supplementations, including blood thinners (anticoagulants such as warfarin, apixaban, rivaroxaban, dabigatran, edoxaban) and antiplatelet agents (such as aspirin, clopidogrel, prasugrel, ticagrelor), can increase the risk of bleeding during and after procedures. Patient was instructed that if they are currently taking any of these medications, OR if they are prescribed any of these medications in the future prior to their scheduled procedure, they must NOT independently stop or continue these medications without explicit guidance from the prescribing physician. Patient was educated that inappropriate management of these medications can result in serious complications including stroke, heart attack, blood clots, or major bleeding.Patient was advised to immediately contact the office if any anticoagulant or antiplatelet medication is started, stopped, or changed prior to the procedure date.Patient was instructed to inform all healthcare providers about their upcoming procedure when discussing any

medication changes. Patient verbalized understanding of these instructions and the importance of coordinating care between all providers.

We discussed with the potential side effects of medication prescribed and urged the patient to read all FDA approved materials that the pharmacy provides with the prescription.***
The above plan and management options were discussed at length with the patient. The patient is in agreement with the above and verbalized understanding.
It will be communicated with the referring physician via electronic record, fax, or mail.

Attestation

Youssef Saweris, MD
5/4/2026
May 4, 2026

Other Notes

All notes

Care Team Note from Miranda, Gina A

Instructions

After Visit Summary (English Snapshot) - Printed 5/4/2026

Additional Documentation

Vitals: BP 131/67(F)

Communications

All Conversations

(Oldest Message First)

Fred Brice to PCC My Health Online Appt Requests

Appointment for: Fred Brice (57554590) 3/12/26 9:50 AM
Visit type: NEW PATIENT/CONSULTATION (15400033)
5/4/2026 10:30 AM (20 minutes) with Youssef Saweris in SGCV PAIN MGMT MODESTO 1401 SPANOS CT STE 130

Patient comments:
Numbness and tingling - Neuroforaminal stenosis of lumbar spine

Sent to (Name not recorded)

Sutter Health: 4/27/26 1:45 PM

Fred, you have an upcoming visit on May 4, 26 at 10:30 AM at 1401 Spanos Court, Suite 130 Modesto CA 95355-2815. Arrive by 10:15 AM. Details: *REDACTED*

Reply 1 to confirm
Reply 2 to reschedule
Reply 3 to cancel

Reply STOP to op

System (Name not recorded)

Sutter Health: 5/1/26 1:04 PM

Fred, you have an upcoming visit on May 4, 26 at 10:30 AM at 1401 Spanos Court, Suite 130 Modesto CA 95355-2815. Arrive by 10:15 AM. Details: *REDACTED*

Reply 1 to confirm
Reply 2 to reschedule
Reply 3 to cancel

Reply STOP to op

System (Name not recorded)

Sutter Health: 5/3/26 1:58 PM

Fred, you have an upcoming visit on May 4, 26 at 10:30 AM at 1401 Spanos Court, Suite 130 Modesto CA 95355-2815. Arrive by 10:15 AM. Details: *REDACTED*

Reply 1 to confirm
Reply 2 to reschedule
Reply 3 to cancel

Reply STOP to op

This encounter is not signed. The conversation may still be ongoing. Notes are filtered from this report.

Orders Placed

None

Medication Changes

As of 5/4/2026 10:40 AM

None

Medication List at End of Visit

As of 5/4/2026 10:40 AM

	Refills	Start Date	End Date
atorvastatin (LIPITOR) 20mg Tab Take one Tab by mouth daily Dose changed - Oral	3	12/18/2025	—
bupROPion ER (WELLBUTRIN XL) 300mg Tab Take one Tab by mouth daily Dose change - Oral	3	10/14/2025	—
diclofenac sodium (VOLTAREN) 1% Gel four g to affected area(s) four times daily (max: 16 grams/joint/day; 32 grams/day body max) - Topical	5	11/10/2025	—
empagliflozin (JARDIANCE) 25mg Tab Take one Tab by mouth every morning Dose change - Oral	3	10/14/2025	—
etodolac (LODINE) 500mg Tab Take one Tab by mouth twice daily - Oral	0	4/15/2026	—
folic acid 1mg Tab Take one Tab by mouth daily - Oral	5	4/15/2026	—

	Refills	Start Date	End Date
gabapentin (NEURONTIN) 300mg Cap	5	12/18/2025	—
Take one Cap by mouth twice daily - Oral			
methotrexate 2.5mg Tab	5	4/15/2026	—
Take four Tab by mouth once weekly - Oral			
omeprazole DR (PRILOSEC) 40mg DR Cap	0	4/15/2026	—
Take one Cap by mouth daily 30 minutes before breakfast - Oral			
pioglitazone (ACTOS) 15mg Tab	3	4/21/2026	—
Take one Tab by mouth daily Dose change - Oral			
sildenafil (VIAGRA) 100mg Tab	11	6/26/2025	—
Take one Tab by mouth as directed Take 30 minutes to 4 hours before sexual activity. Max dose is 100mg in 24 hours. - Oral			
tadalafil (CIALIS) 10mg Tab	11	10/14/2025	—
Take one Tab by mouth as directed Take 30 minutes prior to anticipated sexual activity. Max dose is 20mg in 24 hours. - Oral			
Tirzepatide			
tirzepatide (MOUNJARO) 5mg/0.5mL Auto Injector	0	4/21/2026	—
Administer 0.5 mL subcutaneously once weekly - Subcutaneous			
tirzepatide (MOUNJARO) 7.5mg/0.5mL Auto Injector	0	5/12/2026	7/11/2026
Administer 0.5 mL subcutaneously once weekly Start 7.5 mg weekly after completing 4 weeks of 5 mg weekly - Subcutaneous			

Visit Diagnoses

Primary: **Lumbar radiculopathy** M54.16
Cervical spondylosis M47.812
Cervical radiculopathy M54.12
S/P laminectomy Z98.890